

MedAI Clinical Summary

Patient Interview & SOAP Note Documentation

Date: January 11, 2026 at 08:36 AM

■ SUBJECTIVE

- Patient-reported symptoms and history: Patient presents with a chief complaint of sharp chest pain, rating the severity as 7 out of 10. The pain is located in the chest. Patient also reports experiencing a fever. No further history regarding onset, duration, aggravating or alleviating factors, or associated symptoms is available at this time.

■ OBJECTIVE

- Expected physical exam findings:
- General: Patient is expected to appear uncomfortable and potentially febrile.
- Cardiovascular: Heart sounds may be tachycardic; assessment for pericardial rubs (suggestive of pericarditis) or murmurs is crucial. Peripheral pulses should be strong and symmetrical.
- Respiratory: Lung auscultation may reveal crackles, egophony (suggesting consolidation like pneumonia), or a pleural rub (suggestive of pleurisy). Respiratory rate may be elevated. Palpation of the chest wall for localized tenderness to help rule out musculoskeletal causes like costochondritis.
- Abdominal: Expected to be soft and non-tender to palpation to help rule out referred pain from abdominal etiologies.
- Vital signs: Unavailable at this time. Expected findings given reported symptoms include elevated temperature, potentially increased heart rate, and respiratory rate.

■ ASSESSMENT

- Primary diagnosis: Pleurisy (likely inflammatory or infectious etiology given the sharp pain and fever).
- Differential diagnosis:

- Pneumonia
- Pericarditis
- Costochondritis
- Acute Coronary Syndrome (less likely given the primary presentation of sharp pain and fever, but must be ruled out due to location)
- Pulmonary Embolism (consider if risk factors present, though classic symptoms are not fully described)

■ PLAN

- Tests:
 - Complete Blood Count (CBC) with differential, C-reactive protein (CRP), Erythrocyte Sedimentation Rate (ESR) to assess for inflammation or infection.
 - Troponin I/T to rule out cardiac injury.
 - Electrocardiogram (ECG) for cardiac rhythm and to rule out ischemic changes or pericarditis.
 - Chest X-ray (CXR) to evaluate for pulmonary infiltrates, pleural effusions, or other thoracic pathology.
- Medications:
 - Acetaminophen (Tylenol) or Ibuprofen (NSAID) for symptomatic relief of pain and fever.
 - Empiric broad-spectrum antibiotics may be considered if an infectious etiology like pneumonia is strongly suspected after initial diagnostic tests.
- Patient advice:
 - Emphasize the importance of rest and maintaining adequate hydration.
 - Advise the patient to monitor their symptoms closely and report any worsening of chest pain, shortness of breath, dizziness, or new onset symptoms immediately.
- Follow-up instructions:
 - Recommend a follow-up appointment within 24-48 hours for re-evaluation and review of test results.

- Instruct the patient to present to the emergency department sooner if symptoms escalate or new concerning symptoms develop.

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